

📖 Psychological Rating Scales

Clinical Psychology Cheat Sheet

MPhil Clinical Psychology | Viva | Clinical Postings | Quick Revision

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1. Beck Depression Inventory - Second Edition (BDI-II)

Depression

Purpose / Clinical Use

- Severity rating of depressive symptoms
- Screening for depression in clinical and research settings
- Monitoring treatment response over time
- NOT a diagnostic tool — supports clinical judgment

Structure

- **Items:** 21 items
- **Response format:** 4-point Likert scale (0–3) for each item
- **Subscales:** Cognitive-Affective (items 1–14) & Somatic-Performance (items 15–21)
- **Content areas:** Sadness, pessimism, past failure, loss of pleasure, guilty feelings, punishment feelings, self-dislike, self-criticalness, suicidal thoughts, crying, agitation, loss of interest, indecisiveness, worthlessness, loss of energy, sleep changes, irritability, appetite changes, concentration difficulty, fatigue, loss of interest in sex

Scoring Method

- **Step 1:** Score each item 0–3 based on severity endorsed
- **Step 2:** Sum all 21 items for total score
- **Score range:** 0–63
- **Special rule:** If patient selects two responses for one item, score the HIGHER value
- **Item 16 (Sleep) & Item 18 (Appetite):** Have both increase and decrease options — score highest endorsed

Score Range	Severity
0–13	Minimal Depression

14–19	Mild Depression
20–28	Moderate Depression
29–63	Severe Depression

Clinical Interpretation

- **High scores (29+):** Significant depressive episode; evaluate for suicidality
- **Low scores (0–13):** Minimal symptoms; may indicate remission or subclinical presentation
- **Red flag:** Item 9 (Suicidal ideation) — any score ≥ 1 requires immediate clinical follow-up

Administration Details

- **Type:** Self-report
- **Time:** 5–10 minutes
- **Age:** 13 years and above
- **Time frame assessed:** Past 2 weeks

Advantages

- Quick, easy to administer
- Excellent psychometric properties (high internal consistency $\alpha = 0.92$)
- Aligned with DSM-IV/DSM-5 criteria
- Widely validated across cultures including Indian populations
- Sensitive to change — useful for treatment monitoring

Limitations

- Self-report bias (under/over-reporting)
- Somatic items may inflate scores in medical populations
- Copyrighted (Pearson) — requires purchase
- Not suitable for diagnosis alone
- Cultural interpretation of items may vary

Alternatives / Similar Scales

- **PHQ-9:** 9-item, free, directly maps DSM-5 criteria; better for screening in primary care
- **HAM-D:** Clinician-rated; better for research & severe depression assessment
- **Zung SDS:** Self-report, 20 items; older, less sensitive to change
- **CES-D:** 20-item epidemiological tool; focus on community screening

2. Hamilton Depression Rating Scale (HAM-D / HDRS)

Depression

Purpose / Clinical Use

- Clinician-rated severity assessment of depression
- Gold standard for clinical trials & research
- Monitoring treatment response (pharmacotherapy & psychotherapy)
- Evaluating depressive episodes in already-diagnosed patients

Structure

- **Items:** 17 items (HAM-D-17; original), also available as 21-item & 24-item versions
- **Response format:** Clinician-rated; mix of 0–4 and 0–2 scales
- **9 items:** Scored 0–4 (absent to severe)
- **8 items:** Scored 0–2 (absent, slight/doubtful, clearly present)
- **Key domains:** Depressed mood, guilt, suicide, insomnia (early/middle/late), work/activities, retardation, agitation, anxiety (psychic/somatic), somatic symptoms, genital symptoms, hypochondriasis, weight loss, insight

Scoring Method

- **Step 1:** Rate each item based on clinical interview & observation
- **Step 2:** Sum all 17 items
- **Score range:** 0–52 (17-item version)
- **Note:** Items 18–21 (in 21-item version) assess diurnal variation, depersonalization, paranoid symptoms, obsessional symptoms — NOT included in total score

Score Range (17-item)	Severity
0–7	Normal / Remission
8–13	Mild Depression
14–18	Moderate Depression

19–22	Severe Depression
≤23	Very Severe Depression

Response criteria: ≥50% reduction = Response; Score ≤7 = Remission

Clinical Interpretation

- **High scores (≥23):** Very severe depressive episode; high functional impairment
- **Low scores (≤7):** Clinical remission
- **Red flag:** Item 3 (Suicide) — score of 3 or 4 requires immediate risk assessment
- Emphasis on somatic/vegetative symptoms — may overestimate severity in medically ill

Administration Details

- **Type:** Clinician-rated (semi-structured interview)
- **Time:** 15–20 minutes
- **Age:** Adults (18+)
- **Training required:** Yes, for reliable administration

Advantages

- Gold standard in depression research
- Clinician-rated — less subject to response bias
- Well-established psychometric properties
- Widely used in pharmacological trials
- Free to use

Limitations

- Requires trained clinician — inter-rater reliability concerns
- Heavy emphasis on somatic/insomnia symptoms
- Less sensitive to atypical depression features
- Time-consuming compared to self-report measures
- Uneven item scoring (mix of 3-point and 5-point items)

Alternatives / Similar Scales

- **MADRS:** 10-item clinician-rated; more sensitive to change; less somatic emphasis
- **BDI-II:** Self-report; quicker; patient perspective
- **PHQ-9:** Brief self-report; free; maps to DSM-5 criteria directly
- **QIDS:** 16-item; equal weighting of DSM criterion domains

B. ANXIETY SCALES

3. Beck Anxiety Inventory (BAI)

Anxiety

Purpose / Clinical Use

- Measures severity of anxiety symptoms
- Discriminates anxiety from depression (focuses on somatic/physiological symptoms)
- Monitoring treatment response
- Screening in clinical settings

Structure

- Items:** 21 items
- Response format:** 4-point Likert (0–3): Not at all, Mildly, Moderately, Severely
- Content:** Predominantly somatic/physiological anxiety symptoms (numbness, hot flashes, wobbliness, heart pounding, dizziness, trembling, etc.)
- Time frame:** Past 1 week

Scoring Method

- Step 1:** Rate each item 0–3
- Step 2:** Sum all 21 items
- Score range:** 0–63
- No reverse scoring**

Score Range	Severity
0–7	Minimal Anxiety
8–15	Mild Anxiety
16–25	Moderate Anxiety
26–63	Severe Anxiety

Clinical Interpretation

- **High scores (26+):** Significant somatic anxiety; rule out panic disorder, medical causes
- **Low scores (0–7):** Minimal anxiety; possible remission
- **Note:** May underestimate cognitive/worry-based anxiety (GAD-type)
- **Red flag:** Very high scores with sudden onset — consider panic disorder or medical emergency

Administration Details

- **Type:** Self-report
- **Time:** 5–10 minutes
- **Age:** 17 years and above

Advantages

- Good discriminant validity (anxiety vs. depression)
- Quick and easy
- High internal consistency ($\alpha = 0.92$)
- Useful in panic disorder assessment

Limitations

- Overemphasis on somatic symptoms — may miss cognitive anxiety (worry, apprehension)
- May inflate scores in medical/elderly populations
- Copyrighted (Pearson)
- Less useful for GAD assessment

Alternatives / Similar Scales

- **GAD-7:** Free, 7-item; better for cognitive/worry symptoms of GAD
- **HAM-A:** Clinician-rated; covers both somatic & psychic anxiety
- **STAI:** State-Trait; differentiates current vs. trait anxiety
- **DASS-21 (Anxiety subscale):** Measures anxiety alongside depression & stress

4. Hamilton Anxiety Rating Scale (HAM-A)

Anxiety

Purpose / Clinical Use

- Clinician-rated severity assessment of anxiety
- Monitoring treatment efficacy in anxiety disorders
- Research tool for pharmacological trials
- Assesses both psychic and somatic anxiety

Structure

- **Items:** 14 items
- **Response format:** Clinician-rated, 5-point scale (0–4) per item
- **Two clusters:**
 - Psychic Anxiety (7 items):** Anxious mood, tension, fears, insomnia, intellectual/cognitive, depressed mood, behavior at interview
 - Somatic Anxiety (7 items):** Muscular, sensory, cardiovascular, respiratory, GI, genitourinary, autonomic symptoms

Scoring Method

- **Step 1:** Rate each item 0 (not present) to 4 (very severe/incapacitating)
- **Step 2:** Sum all 14 items
- **Score range:** 0–56
- **Subscale scores:** Psychic anxiety (items 1–6, 14) + Somatic anxiety (items 7–13)

Score Range	Severity
<17	Mild Anxiety
18–24	Mild to Moderate Anxiety
25–30	Moderate to Severe Anxiety
≥31	Very Severe Anxiety

Response criteria: $\geq 50\%$ reduction in score = Treatment response

Clinical Interpretation

- **High scores (≥ 31):** Severe, incapacitating anxiety; significant functional impairment
- **Low scores (< 17):** Mild anxiety; may not require intensive intervention
- **Psychic > Somatic:** Suggests generalized anxiety/worry pattern
- **Somatic > Psychic:** Suggests prominent physical anxiety; rule out medical causes

Administration Details

- **Type:** Clinician-rated (semi-structured interview)
- **Time:** 10–15 minutes
- **Age:** Adults (18+)

Advantages

- Comprehensive coverage of both psychic & somatic anxiety
- Gold standard for clinical trials
- Free to use
- Good sensitivity to treatment change

Limitations

- Overlap with depression symptoms (Item 6 = depressed mood)
- Requires trained clinician
- Poor discriminant validity between anxiety & depression
- Developed for GAD; less specific for panic/social anxiety/OCD

Alternatives / Similar Scales

- **BAI:** Self-report; better discriminant validity from depression; somatic focus
- **GAD-7:** Brief, free, self-report; specific to GAD
- **STAI:** Differentiates state vs. trait anxiety
- **Liebowitz Social Anxiety Scale:** Specific for social phobia

C. PSYCHOSIS SCALES

5. Brief Psychiatric Rating Scale (BPRS)

Psychosis / General Psychopathology

Purpose / Clinical Use

- Assesses severity of general psychiatric symptoms
- Monitoring treatment response in psychotic disorders
- Quick overall psychopathology assessment
- Widely used in inpatient and research settings

Structure

- Items:** 18 items (expanded version: 24 items)
- Response format:** Clinician-rated, 7-point scale (1–7) per item
- 1 = Not present, 2 = Very mild, 3 = Mild, 4 = Moderate, 5 = Moderately severe, 6 = Severe, 7 = Extremely severe
- Domains covered:** Somatic concern, anxiety, emotional withdrawal, conceptual disorganization, guilt, tension, mannerisms, grandiosity, depressed mood, hostility, suspiciousness, hallucinatory behavior, motor retardation, uncooperativeness, unusual thought content, blunted affect, excitement, disorientation

Scoring Method

- Step 1:** Rate each item 1–7 based on interview + observation
- Step 2:** Sum all 18 items for total score
- Score range:** 18–126 (18-item version)
- Note:** Minimum score is 18 (not 0), since lowest rating per item = 1

Score Range	Severity
18–31	Minimal / Not Ill
32–40	Mildly Ill
41–52	Moderately Ill

53–65

Markedly III

≥66

Severely III

Clinical Interpretation

- **High scores (≥53):** Significant psychopathology; likely needs inpatient care
- **Low scores (≤31):** Minimal symptoms; possible remission
- **Pattern analysis:** Look at individual item clusters for positive symptoms vs. negative symptoms vs. affective symptoms

Administration Details

- **Type:** Clinician-rated
- **Time:** 15–20 minutes
- **Age:** Adults (18+)
- **Based on:** Clinical interview + behavioral observation

Advantages

- Brief and comprehensive
- Covers wide range of psychopathology (not limited to psychosis)
- Good inter-rater reliability with training
- Free to use
- Useful in acute settings

Limitations

- Does not differentiate positive from negative symptoms as well as PANSS
- Requires trained rater
- Limited assessment of negative symptoms
- Less specific than PANSS for schizophrenia research

Alternatives / Similar Scales

- **PANSS:** 30-item; separate positive, negative & general subscales; more detailed for schizophrenia
- **CGI:** 1-item global severity; ultra-brief clinical impression

- **SAPS/SANS:** Separate scales for positive & negative symptoms; more detailed item-level assessment

6. Positive and Negative Syndrome Scale (PANSS)

Schizophrenia / Psychosis

Purpose / Clinical Use

- Comprehensive assessment of schizophrenia symptom dimensions
- Differentiates positive, negative, and general psychopathology
- Gold standard for schizophrenia research & drug trials
- Monitoring treatment response

Structure

- **Items:** 30 items total
- **3 Subscales:**
 - Positive Scale (P): 7 items** — Delusions, conceptual disorganization, hallucinatory behavior, excitement, grandiosity, suspiciousness, hostility
 - Negative Scale (N): 7 items** — Blunted affect, emotional withdrawal, poor rapport, passive/apathetic social withdrawal, difficulty in abstract thinking, lack of spontaneity, stereotyped thinking
 - General Psychopathology (G): 16 items** — Somatic concern, anxiety, guilt, tension, mannerisms, depression, motor retardation, uncooperativeness, unusual thought content, disorientation, poor attention, lack of judgment, disturbance of volition, poor impulse control, preoccupation, active social avoidance
- **Response format:** 7-point scale (1–7) per item

Scoring Method

- **Step 1:** Rate each item 1–7 based on 30–40 min structured interview (SCI-PANSS)
- **Step 2:** Calculate subscale scores:
 - Positive Scale: Sum P1–P7 (Range: 7–49)
 - Negative Scale: Sum N1–N7 (Range: 7–49)
 - General: Sum G1–G16 (Range: 16–112)
- **Total PANSS Score:** P + N + G (Range: 30–210)

- **Composite Index:** P minus N (indicates predominance of positive vs. negative symptoms)

Subscale	Range	Interpretation
Positive (P)	7–49	Higher = more positive symptoms
Negative (N)	7–49	Higher = more negative symptoms
General (G)	16–112	Higher = more general psychopathology
Total	30–210	Overall illness severity

Total Score	Approximate Severity
30–58	Mildly Ill
59–75	Moderately Ill
76–95	Markedly Ill
≥96	Severely Ill

Response: ≥20% reduction in total PANSS = minimal response; ≥50% = good response

Clinical Interpretation

- **P > N:** Predominant positive symptoms; better prognosis; more responsive to typical antipsychotics
- **N > P:** Predominant negative symptoms; poorer prognosis; consider atypical antipsychotics
- **High G scores:** Significant general psychopathology beyond core schizophrenia symptoms

Administration Details

- **Type:** Clinician-rated (requires SCI-PANSS structured interview)
- **Time:** 30–40 minutes
- **Age:** Adults (18+)
- **Training:** Required; certification recommended for research

Advantages

- Most comprehensive scale for schizophrenia
- Clear separation of symptom dimensions
- Excellent psychometric properties
- Standard in clinical trials worldwide
- Composite index useful for subtype classification

Limitations

- Time-consuming (30–40 minutes)
- Requires trained rater & structured interview
- Copyrighted
- Negative scale has been criticized for including cognitive items
- May not capture full deficit syndrome

Alternatives / Similar Scales

- **BPRS:** Shorter (18 items); quicker; less specific for schizophrenia
- **SAPS/SANS:** Separate detailed scales for positive/negative symptoms; more item granularity
- **CGI-SCH:** Brief global impression specific to schizophrenia
- **PSP:** Personal and Social Performance scale; focuses on functioning rather than symptoms

7. Scale for the Assessment of Positive Symptoms (SAPS)

Psychosis – Positive Symptoms

Purpose / Clinical Use

- Detailed assessment of positive symptoms in schizophrenia
- Complements SANS for comprehensive symptom profiling
- Research tool; used in longitudinal monitoring

Structure

- **Items:** 34 items + 4 global rating items = 34 items grouped into 4 domains
- **4 Domains (with global ratings):**
 1. Hallucinations (7 items + 1 global)
 2. Delusions (13 items + 1 global)
 3. Bizarre Behavior (4 items + 1 global)
 4. Positive Formal Thought Disorder (9 items + 1 global)
- **Response format:** 6-point scale (0–5) per item
- 0 = None, 1 = Questionable, 2 = Mild, 3 = Moderate, 4 = Marked, 5 = Severe

Scoring Method

- **Step 1:** Rate each item 0–5
- **Step 2:** Calculate domain scores by summing items within each domain
- **Step 3:** Total score = Sum of all items OR sum of 4 global ratings (Summary score)
- **Summary score (4 global ratings):** Range 0–20
- **Total composite score:** Range 0–170 (all individual items)
- **No formal severity cut-offs published** — interpret relative to baseline & norms

Domain	No. of Items	Score Range
Hallucinations	7	0–35
Delusions	13	0–65

Bizarre Behavior	4	0–20
Positive Formal Thought Disorder	9	0–45

Clinical Interpretation

- **Higher scores:** More prominent positive symptoms; florid psychosis
- **Domain-specific analysis:** Helps identify predominant symptom type (e.g., hallucination-predominant vs. delusion-predominant)
- **Change scores:** Most useful for tracking treatment response over time

Administration Details

- **Type:** Clinician-rated
- **Time:** 20–30 minutes (often combined with SANS)
- **Age:** Adults
- **Developer:** Nancy Andreasen (1984)

Advantages

- Very detailed item-level assessment of positive symptoms
- Includes formal thought disorder assessment
- Free to use
- Good for research on specific symptom dimensions

Limitations

- No established severity cut-offs
- Time-consuming when combined with SANS
- Less commonly used in clinical practice (PANSS preferred)
- Requires clinical expertise for rating

Alternatives / Similar Scales

- **PANSS Positive subscale:** Shorter; part of comprehensive PANSS; established cut-offs
- **BPRS:** Briefer overall psychopathology assessment

- **PSYRATS:** Specific to hallucinations and delusions; dimensional assessment

8. Scale for the Assessment of Negative Symptoms (SANS)

Psychosis – Negative Symptoms

Purpose / Clinical Use

- Detailed assessment of negative symptoms in schizophrenia
- Complements SAPS for full symptom profiling
- Research & clinical monitoring of deficit states
- Evaluating response to atypical antipsychotics

Structure

- **Items:** 25 items grouped into 5 domains (+ 5 global ratings)
- **5 Domains:**
 - 1. Affective Flattening/Blunting (8 items + 1 global)
 - 2. Alogia (5 items + 1 global)
 - 3. Avolition/Apathy (4 items + 1 global)
 - 4. Anhedonia/Asociality (5 items + 1 global)
 - 5. Attention (3 items + 1 global)
- **Response format:** 6-point scale (0–5)
- 0 = None, 1 = Questionable, 2 = Mild, 3 = Moderate, 4 = Marked, 5 = Severe

Scoring Method

- **Step 1:** Rate each item 0–5
- **Step 2:** Calculate domain scores (sum items within domain)
- **Step 3:** Summary score = Sum of 5 global ratings (Range: 0–25)
- **Total composite score:** Sum of all 25 items (Range: 0–125)

Domain	Items	Range
Affective Flattening	8	0–40

Alogia	5	0–25
Avolition/Apathy	4	0–20
Anhedonia/Asociality	5	0–25
Attention	3	0–15

Note: No universally established severity cut-offs; interpret relative to baseline and clinical context.

Clinical Interpretation

- **Higher scores:** More prominent negative/deficit symptoms; poorer prognosis
- **Domain patterns:** Helps plan targeted rehabilitation (e.g., high avolition → behavioral activation)
- **Distinguish:** Primary negative symptoms vs. secondary (due to medication, depression, social deprivation)

Administration Details

- **Type:** Clinician-rated (interview + observation)
- **Time:** 15–20 minutes
- **Age:** Adults
- **Developer:** Nancy Andreasen (1984)

Advantages

- Most detailed scale for negative symptoms
- 5 clearly defined domains
- Free to use
- Good reliability with training
- Useful for rehabilitation planning

Limitations

- No formal severity cut-offs
- Attention subscale may overlap with cognitive deficits
- Does not distinguish primary vs. secondary negative symptoms

- Less used in clinical trials now (PANSS-N preferred)

Alternatives / Similar Scales

- **PANSS Negative subscale:** Shorter; part of PANSS; more widely used in trials
- **BNSS (Brief Negative Symptom Scale):** 13-item; newer; better construct validity
- **CAINS:** Motivation/Pleasure + Expression domains; aligned with current negative symptom models
- **NSA-16:** 16-item; clinician-rated; balances brevity & comprehensiveness

D. MANIA SCALE

9. Young Mania Rating Scale (YMRS)

Mania / Bipolar Disorder

Purpose / Clinical Use

- Assesses severity of manic symptoms
- Monitoring treatment response in bipolar disorder (manic/mixed episodes)
- Standard outcome measure in mania clinical trials

Structure

- Items:** 11 items
- Response format:** Clinician-rated
- Scoring:** 7 items scored 0–4; 4 items scored 0–8 (double-weighted items)
- Double-weighted items (0–8):** Irritability, Speech, Thought Content, Disruptive/Aggressive Behavior
- Regular items (0–4):** Elevated mood, Increased motor activity, Sexual interest, Sleep, Appearance, Insight, Language-thought disorder

Scoring Method

- Step 1:** Rate each item based on clinical interview + observation
- Step 2:** 7 items scored 0–4; 4 items scored 0–8
- Step 3:** Sum all items
- Score range:** 0–60
- Important:** Double-weighted items given extra weight due to clinical significance and poor cooperation potential

Score Range	Severity
0–6	Not significant / Remission
7–12	Mild Mania
13–19	Moderate Mania

20–25

Moderate-Severe Mania

≥26

Severe Mania

Response: ≥50% reduction = Treatment response; Score ≥12 = Remission (commonly used criterion)

Clinical Interpretation

- **High scores (≥26):** Severe manic episode; likely requires hospitalization
- **Low scores (≤6):** Remission or euthymic state
- **Red flags:** High irritability + disruptive behavior scores = risk of aggression; High thought content score = psychotic mania
- **Note:** Does not assess depressive symptoms — use with depression scale for mixed states

Administration Details

- **Type:** Clinician-rated
- **Time:** 15–30 minutes
- **Age:** Adults (18+); adapted versions for adolescents available
- **Basis:** Interview + observation + informant report (last 48 hours)

Advantages

- Gold standard for manic episode severity
- Quick and reliable
- Good sensitivity to change
- Free to use
- Double-weighting captures disruptive mania well

Limitations

- No assessment of depressive symptoms (problem in mixed states)
- Requires clinician training
- Limited psychometric data for hypomania
- Double-weighted items may bias toward irritable mania

Alternatives / Similar Scales

- **Bech-Rafaelsen Mania Scale (MAS):** 11-item; equal weighting; more psychometrically balanced
- **Altman Self-Rating Mania Scale (ASRM):** 5-item self-report; quick screening
- **CARS-M:** Clinician-Administered Rating Scale for Mania; distinguishes classic vs. psychotic mania
- **MDQ (Mood Disorder Questionnaire):** Self-report screening for bipolar disorder (not severity)

E. OCD SCALE

10. Yale-Brown Obsessive Compulsive Scale (Y-BOCS)

Obsessive-Compulsive Disorder

Purpose / Clinical Use

- Gold standard for OCD severity assessment
- Rates severity independent of type/content of obsessions/compulsions
- Monitoring treatment response (CBT and pharmacotherapy)
- Research outcome measure

Structure

- **Items:** 10 core items (+ optional investigation/insight items)
- **Two subscales (5 items each):**
 - Obsession Subscale (items 1–5):** Time occupied, interference, distress, resistance, control
 - Compulsion Subscale (items 6–10):** Time occupied, interference, distress, resistance, control
- **Response format:** Clinician-rated, 5-point scale (0–4) per item
- **Preceded by:** Y-BOCS Symptom Checklist (64+ symptoms) to identify target symptoms

Scoring Method

- **Step 1:** Administer Y-BOCS Symptom Checklist to identify current obsessions & compulsions
- **Step 2:** Rate items 1–5 for obsessions (0–4 each)
- **Step 3:** Rate items 6–10 for compulsions (0–4 each)
- **Step 4:** Calculate:
 - Obsession subtotal = Sum items 1–5 (Range: 0–20)
 - Compulsion subtotal = Sum items 6–10 (Range: 0–20)
 - Total Y-BOCS = Obsession + Compulsion subtotals (Range: 0–40)**
- **Special note:** If patient has only obsessions (no compulsions), multiply obsession score × 2 for total equivalent

Total Score	Severity
0–7	Subclinical
8–15	Mild OCD
16–23	Moderate OCD
24–31	Severe OCD
32–40	Extreme OCD

Response criteria: ≥35% reduction = Responder; ≥25% = Partial response; Score ≤12 = Remission (commonly used)

Clinical Interpretation

- **High scores (24+):** Severe OCD; significant functional impairment; may need combined CBT + SSRI
- **Low scores (≤7):** Subclinical; obsessions/compulsions not clinically significant
- **Obsession > Compulsion score:** Predominantly obsessional OCD (ruminations/mental rituals)
- **Red flag:** Item 3 (distress) very high with low resistance (item 4) = poor prognosis without treatment

Administration Details

- **Type:** Clinician-rated (semi-structured interview)
- **Time:** 15–30 minutes (including symptom checklist)
- **Age:** Adults; CY-BOCS for children (ages 6–17)

Advantages

- Gold standard for OCD severity
- Content-independent (rates severity regardless of OCD type)
- Excellent psychometric properties
- Widely validated across cultures
- Free to use

Limitations

- Requires trained clinician
- Does not assess insight (Item 11 optional, not in total)
- May not capture avoidance behaviors well
- Compulsion score may be 0 for purely obsessional OCD (use multiply rule)

Alternatives / Similar Scales

- **OCI-R (Obsessive-Compulsive Inventory-Revised)**: Self-report; 18 items; assesses OCD symptom dimensions
- **DOCS (Dimensional OCD Scale)**: Self-report; 20 items; 4 empirically-based dimensions
- **CY-BOCS**: Child/adolescent version; same structure adapted for ages 6–17
- **BABS**: Assesses degree of conviction/insight in beliefs (used alongside Y-BOCS)

F. COGNITIVE ASSESSMENT SCALES

11. Mini Mental State Examination (MMSE)

Cognitive Functioning / Dementia

Purpose / Clinical Use

- Brief screening for cognitive impairment
- Detecting dementia and delirium
- Monitoring cognitive decline over time
- Most widely used bedside cognitive screen globally

Structure

- **Items:** 11 items (30 points total)
- **Domains assessed:**
 - Orientation to Time (5 points)
 - Orientation to Place (5 points)
 - Registration (3 points)
 - Attention & Calculation (5 points) — Serial 7s or WORLD backwards
 - Recall (3 points)
 - Language (8 points) — Naming, repetition, 3-stage command, reading, writing
 - Visuospatial (1 point) — Copy intersecting pentagons

Scoring Method

- **Step 1:** Score each item correct (1) or incorrect (0)
- **Step 2:** Sum all points
- **Total score range:** 0–30
- **Higher score = better cognition**

Score	Interpretation
27–30	Normal
24–26	Mild Cognitive Impairment

18–23	Mild Dementia
10–17	Moderate Dementia
<10	Severe Dementia

Adjust for education: In Indian populations, cut-off of 24 may be too high for illiterate/low-education groups. Cut-off of 23 or lower often used. Some use 25/26 for educated populations.

Clinical Interpretation

- **Score <24:** Generally indicates cognitive impairment requiring further evaluation
- **≥3 point decline over 6 months:** Clinically significant deterioration
- **Pattern analysis:** Orientation loss early = Alzheimer's; Attention loss = Delirium
- **Red flag:** Rapid decline (>5 points in 6 months) = consider delirium, rapidly progressive dementia

Administration Details

- **Type:** Clinician-administered
- **Time:** 5–10 minutes
- **Age:** Adults (generally 60+, but usable from 18+)

Advantages

- Quick bedside administration
- Universally known and recognized
- Good for serial monitoring
- Hindi and regional language versions available for India

Limitations

- Copyrighted (PAR Inc.) since 2001
- Ceiling effect — misses MCI and early dementia in educated individuals
- Floor effect in advanced dementia
- No executive function assessment
- Education/literacy bias (especially in Indian context)
- Poor sensitivity for frontal/subcortical dementias

Alternatives / Similar Scales

- **MoCA:** Better sensitivity for MCI; includes executive function; free; 30 points
- **ACE-III:** More comprehensive (100 points); covers 5 cognitive domains; better for differentiating dementia types
- **HMSE (Hindi Mental State Examination):** Adapted for Indian illiterate populations
- **CDR (Clinical Dementia Rating):** Stages dementia severity based on 6 domains; informant-based

12. Montreal Cognitive Assessment (MoCA)

Cognitive Functioning / MCI

Purpose / Clinical Use

- Screening for Mild Cognitive Impairment (MCI)
- More sensitive than MMSE for early/mild cognitive deficits
- Assesses executive functions (unlike MMSE)
- Useful across multiple neurological conditions (PD, stroke, TBI)

Structure

- **Items:** Multiple tasks, total 30 points
- **Domains assessed (8 cognitive areas):**
 - Visuospatial/Executive (5 points) — Trail Making B adaptation, cube copy, clock drawing
 - Naming (3 points) — 3 animals
 - Attention (6 points) — Digit span, vigilance, serial 7s
 - Language (3 points) — Repetition, fluency
 - Abstraction (2 points)
 - Delayed Recall (5 points) — 5 words
 - Orientation (6 points)

Scoring Method

- **Step 1:** Score each task as per manual instructions
- **Step 2:** Sum all domain scores
- **Step 3:** Add 1 point if education $\leq$ 12 years (education correction)
- **Total score range:** 0–30
- **Higher = better cognition**

Score	Interpretation
$\geq$ 26	Normal

18–25	Mild Cognitive Impairment
10–17	Moderate Cognitive Impairment
<10	Severe Cognitive Impairment

Cut-off: <26 = Cognitive impairment (original). Some studies suggest <23 for Indian populations with lower education.

Clinical Interpretation

- **Score <26:** Screen positive for cognitive impairment; needs detailed neuropsychological assessment
- **Domain-specific deficits:** Help localize impairment (e.g., low executive = frontal; low recall = temporal/hippocampal)
- **Differentiating patterns:** Alzheimer's = low delayed recall; Vascular = low executive/attention; Lewy body = visuospatial deficits

Administration Details

- **Type:** Clinician-administered
- **Time:** 10–15 minutes
- **Age:** Adults (55+; usable from 18+)
- **Available in:** Multiple languages including Hindi
- **Website:** www.mocatest.org (free for clinical use with certification)

Advantages

- Superior to MMSE for MCI detection (sensitivity 90% vs 18% for MMSE)
- Assesses executive function
- Free for clinical use (with online certification)
- Multiple parallel versions available (reduces practice effects)
- Available in 100+ languages

Limitations

- Education bias (even with +1 correction)
- Higher false positive rate in low-education populations
- Ceiling effect in highly educated individuals

- Requires certification for use (since 2019)
- Less studied in Indian illiterate populations compared to HMSE

Alternatives / Similar Scales

- **MMSE:** Simpler; less sensitive for MCI; no executive function; more widely known
- **ACE-III:** More comprehensive (100 points); better domain profiling; longer administration
- **RUDAS:** Cross-cultural cognitive screen; less education bias; 6 items
- **Mini-ACE:** 5-minute version of ACE; 30 points; good balance

13. Addenbrooke's Cognitive Examination (ACE-III / ACE-R)

Cognitive Functioning / Dementia Differentiation

Purpose / Clinical Use

- Comprehensive bedside cognitive assessment
- Differentiating types of dementia (Alzheimer's vs. Frontotemporal)
- More detailed than MMSE/MoCA while remaining clinically practical
- Detecting early cognitive decline across 5 domains

Structure

- **Items:** Multiple tasks, total 100 points
- **5 Cognitive Domains:**
 - Attention & Orientation (18 points)
 - Memory (26 points)
 - Fluency (14 points) — Letter & category
 - Language (26 points)
 - Visuospatial (16 points)
- **Note:** MMSE items are embedded within ACE-III (can derive MMSE score)

Scoring Method

- **Step 1:** Score each task as per manual
- **Step 2:** Calculate domain totals
- **Step 3:** Sum all domains for total ACE-III score (0–100)
- **VLOM Ratio:** $(V + L) / (O + M)$ — helps differentiate AD from FTD

Score (ACE-III)	Interpretation
≥88	Normal (high sensitivity cut-off)
82–87	Possible impairment (investigate further)

<82

Likely dementia (high specificity cut-off)

Cut-off	Sensitivity	Specificity
88	94%	89%
82	84%	100%

VLOM Ratio: >3.2 suggests Alzheimer's; <2.2 suggests Frontotemporal Dementia;
2.2–3.2 = indeterminate

Clinical Interpretation

- **Alzheimer's pattern:** Low memory + low orientation; relatively preserved language
- **FTD pattern:** Low fluency + low language; relatively preserved memory & visuospatial
- **Domain profile more informative than total score alone**

Administration Details

- **Type:** Clinician-administered
- **Time:** 15–20 minutes
- **Age:** Adults (typically 50+)
- **Free to use** (available online)

Advantages

- Most comprehensive bedside cognitive test
- VLOM ratio for dementia differentiation
- Free and publicly available
- 5 domain scores provide cognitive profile
- Embeds MMSE (backward compatible)

Limitations

- Longer administration than MMSE/MoCA
- Education and language bias
- Less validation in Indian populations compared to MMSE

- Ceiling effect in highly educated younger adults

Alternatives / Similar Scales

- **MoCA:** Shorter (10 min); 30 points; better for MCI screening; less domain detail
- **MMSE:** Shorter (5–10 min); 30 points; less comprehensive; widely available
- **DRS-2 (Dementia Rating Scale):** 36-item; 144 points; similar domain coverage; normed
- **ADAS-Cog:** Gold standard for AD clinical trials; more detailed memory assessment

G. CHILD & ADOLESCENT SCALES

14. Child Behavior Checklist (CBCL)

Child Behavior / Psychopathology

Purpose / Clinical Use

- Comprehensive assessment of behavioral/emotional problems in children
- Part of ASEBA (Achenbach System of Empirically Based Assessment)
- Screening, clinical assessment, treatment monitoring
- Parent/caregiver report of child's behavior

Structure

- **Versions:** CBCL/1.5–5 (preschool) & CBCL/6–18 (school-age)
- **Items:** 113 items (CBCL/6–18); 99 items (CBCL/1.5–5)
- **Response format:** 3-point scale: 0 = Not true, 1 = Somewhat true, 2 = Very/Often true
- **Broadband scales:**
 - Internalizing:** Anxious/Depressed, Withdrawn/Depressed, Somatic Complaints
 - Externalizing:** Rule-Breaking Behavior, Aggressive Behavior
- **Syndrome scales (8):** Anxious/Depressed, Withdrawn/Depressed, Somatic Complaints, Social Problems, Thought Problems, Attention Problems, Rule-Breaking, Aggressive
- **DSM-oriented scales (6):** Affective, Anxiety, Somatic, ADHD, Oppositional Defiant, Conduct Problems

Scoring Method

- **Step 1:** Sum items for each syndrome scale
- **Step 2:** Convert raw scores to T-scores using normative tables (age & gender specific)
- **Step 3:** Interpret T-scores:

T-Score Range	Syndrome Scales	Broadband Scales
<65	Normal	Normal

65–69	Borderline Clinical	Borderline Clinical
70	Clinical Range	Clinical Range

Note: For broadband (Internalizing/Externalizing/Total), clinical cut-off is $T \geq 64$; for syndrome scales, $T \geq 70$.

Clinical Interpretation

- **Internalizing > Externalizing:** Emotional/anxiety/depressive issues predominate
- **Externalizing > Internalizing:** Behavioral/conduct/ADHD issues predominate
- **High Total Problems T-score:** Pervasive psychopathology; multi-domain impairment
- **Cross-informant comparison:** Compare CBCL (parent) with TRF (teacher) and YSR (self-report)

Administration Details

- **Type:** Parent/caregiver report
- **Time:** 15–20 minutes
- **Age:** 1.5–5 years OR 6–18 years
- **Scoring:** Computer scoring recommended (ASEBA software)

Advantages

- Comprehensive multi-dimensional assessment
- Empirically derived scales
- Multi-informant system (CBCL + TRF + YSR)
- Excellent psychometric properties
- Widely used internationally including India

Limitations

- Copyrighted and expensive
- Informant bias (parent perspective only)
- Requires computer scoring for T-scores
- Limited Indian norms (use with caution)
- Does not diagnose — screening/dimensional only

Alternatives / Similar Scales

- **SDQ (Strengths & Difficulties Questionnaire):** 25-item; free; brief; widely used in India; 5 subscales
- **Conners:** Specifically for ADHD assessment; multiple versions
- **BASC-3:** Similar multi-dimensional; also includes adaptive scales
- **PSC (Pediatric Symptom Checklist):** 35-item; brief screening; free

15. Conners Rating Scales (CRS-3 / Conners-3)

ADHD / Child Behavior

Purpose / Clinical Use

- Assessment of ADHD and related problems in children/adolescents
- Differentiation of ADHD subtypes (Inattentive, Hyperactive-Impulsive, Combined)
- Treatment monitoring
- Multi-informant: Parent, Teacher, Self-Report forms

Structure

- **Conners-3 (Full length):**
Parent form: 110 items
Teacher form: 115 items
Self-report (8–18 yrs): 99 items
- **Short forms available:** 43–45 items
- **Response format:** 4-point Likert: 0 = Not at all, 1 = Just a little, 2 = Pretty much, 3 = Very much
- **Content scales:** Inattention, Hyperactivity/Impulsivity, Learning Problems, Executive Functioning, Aggression, Peer Relations
- **DSM-5 Symptom Scales:** ADHD Inattentive, ADHD Hyperactive-Impulsive, Conduct Disorder, Oppositional Defiant Disorder
- **Validity scales:** Positive Impression, Negative Impression, Inconsistency Index

Scoring Method

- **Step 1:** Sum items for each scale
- **Step 2:** Convert raw scores to T-scores (age- & gender-normed)
- **Step 3:** Interpret T-scores

T-Score	Interpretation
≤59	Average (no concerns)

60–64	High Average (possible concern)
65–69	Elevated (probable clinical significance)
≥70	Very Elevated (highly atypical; clinically significant)

ADHD Index: Probability score; higher = more likely ADHD diagnosis. T ≥ 70 strongly suggestive.

Clinical Interpretation

- **Inattention T ≥ 70 + Hyperactivity T < 65:** ADHD Predominantly Inattentive
- **Hyperactivity T ≥ 70 + Inattention T < 65:** ADHD Predominantly Hyperactive-Impulsive
- **Both elevated:** ADHD Combined Presentation
- **Cross-informant discrepancies:** Common in ADHD; explore situational factors

Administration Details

- **Type:** Parent/Teacher/Self-report (multi-informant)
- **Time:** 20 minutes (full); 10 minutes (short)
- **Age:** 6–18 years (parent/teacher); 8–18 years (self-report)
- **Also available:** Conners Adult ADHD Rating Scale (CAARS) for adults

Advantages

- Gold standard for ADHD assessment in children
- Multi-informant system
- DSM-5 aligned symptom scales
- Built-in validity indices
- Normative data extensive

Limitations

- Copyrighted and expensive (MHS publishing)
- Limited Indian norms
- Lengthy full-form administration
- Requires computer scoring for accurate T-scores

- Informant bias possible

Alternatives / Similar Scales

- **Vanderbilt ADHD Scale:** Free; DSM-based; parent & teacher versions; popular in India
- **SNAP-IV:** 26-item; free; maps DSM criteria for ADHD/ODD
- **CBCL (Attention Problems scale):** Part of broader behavioral assessment
- **ADHD-RS (ADHD Rating Scale-5):** 18-item; directly maps DSM-5 ADHD criteria; parent/teacher

16. Indian Scale for Assessment of Autism (ISAA)

Autism Spectrum Disorder (India-specific)

Purpose / Clinical Use

- **Mandated by Government of India** for certification of autism as disability under RPWD Act 2016
- Screening and severity rating of autism in Indian context
- Used for disability certification and benefit eligibility
- Developed by NIMHANS, Bangalore (2009)

Structure

- **Items:** 40 items
- **6 Domains:**
 1. Social Relationship & Reciprocity (10 items)
 2. Emotional Responsiveness (4 items)
 3. Speech-Language & Communication (7 items)
 4. Behavior Patterns (8 items)
 5. Sensory Aspects (6 items)
 6. Cognitive Component (5 items)
- **Response format:** 5-point rating: 1 = Never, 2 = Rarely, 3 = Sometimes, 4 = Often, 5 = Always

Scoring Method

- **Step 1:** Rate each item 1–5 based on observation & parent interview
- **Step 2:** Sum all 40 items
- **Score range:** 40–200
- **Higher score = more severe autism features**

Total Score	Severity / Interpretation
<70	No Autism / Below threshold

70–106	Mild Autism
107–153	Moderate Autism
≥154	Severe Autism

Disability certification: Score ≥70 qualifies for autism disability certificate in India (40%+ disability). Severity determines benefit level.

Clinical Interpretation

- **Score ≥70:** Meets threshold for autism; eligible for disability certification
- **Domain analysis:** Identifies specific areas of difficulty for intervention planning
- **High Social Relationship scores:** Core social deficits prominent
- **High Behavior Pattern scores:** Restrictive/repetitive behaviors prominent

Administration Details

- **Type:** Professional-rated (psychologist/psychiatrist) based on observation + parent interview
- **Time:** 20–30 minutes
- **Age:** 3–22 years (primarily children/adolescents)
- **Required by:** District Disability Boards in India

Advantages

- India-specific; culturally appropriate
- Government-mandated — essential for disability certification
- Covers sensory & cognitive domains (beyond DSM criteria)
- Free to use (government tool)
- Available in multiple Indian languages

Limitations

- Not internationally validated outside India
- Psychometric properties debated
- May not align perfectly with DSM-5/ICD-11 ASD criteria
- Scoring can be subjective without structured guidelines

- Limited use for high-functioning/Asperger's presentations

Alternatives / Similar Scales

- **CARS-2 (Childhood Autism Rating Scale):** 15-item; international gold standard for autism severity; clinician-rated
- **ADOS-2:** Observational assessment; gold standard for diagnosis; requires extensive training
- **ADI-R:** Semi-structured parent interview; diagnostic; research standard
- **SCQ (Social Communication Questionnaire):** 40-item screening; parent-report; derived from ADI-R

H. SUBSTANCE USE & ADDICTION SCALES

17. Alcohol Use Disorders Identification Test (AUDIT)

Alcohol Use / Screening

Purpose / Clinical Use

- Screening for hazardous and harmful alcohol use
- Identifying alcohol use disorders (at-risk drinking)
- Developed by WHO for primary care settings
- Brief intervention decision tool (AUDIT-based stepped care)

Structure

- **Items:** 10 items
- **3 Domains:**
 - Hazardous Alcohol Use (items 1–3): Frequency, quantity, binge drinking
 - Dependence Symptoms (items 4–6): Impaired control, morning drinking, guilt, blackouts
 - Harmful Alcohol Use (items 7–10): Others concerned, injury, failure to fulfill obligations
- **Response format:** 5-point scale (0–4) per item
- **Items 9 & 10:** Scored 0, 2, or 4 only (3 options)

Scoring Method

- **Step 1:** Score each item 0–4
- **Step 2:** Sum all 10 items
- **Score range:** 0–40

Score	Risk Level	Intervention
0–7	Low risk	Alcohol education
8–15	Hazardous use	Simple advice / Brief intervention
16–19	Harmful use	Brief intervention + continued monitoring

20–40

Possible dependence

Referral for specialist assessment & treatment

AUDIT-C (items 1–3 only): Quick 3-item screen. Cut-off: ≥ 4 (men), ≥ 3 (women) = positive screen.

Clinical Interpretation

- **Score 8+:** Warrants further assessment; hazardous drinking pattern
- **Score 20+:** Likely alcohol dependence; needs specialist referral
- **High items 4–6:** Dependence features present
- **High items 7–10:** Harmful consequences already occurring

Administration Details

- **Type:** Self-report or interview-based
- **Time:** 2–5 minutes
- **Age:** Adults (18+)
- **Setting:** Primary care, emergency departments, psychiatric clinics

Advantages

- WHO-developed; internationally validated
- Free to use
- Very brief
- Covers full spectrum (hazardous to dependent use)
- Linked to intervention framework (WHO AUDIT manual)
- Available in Hindi and other Indian languages

Limitations

- Self-report — underreporting common
- Cultural drinking norms may affect interpretation
- Does not assess other substances
- Standard drink definition varies across countries

Alternatives / Similar Scales

- **CAGE:** 4-item; ultra-brief; 2+ positive = alcoholism screen; less sensitive for hazardous drinking
- **MAST:** 25-item; detailed; older; more for dependence detection
- **ASSIST:** Multi-substance; broader coverage; also WHO-developed
- **AUDIT-C:** First 3 items of AUDIT; ultra-brief screening version

18. Alcohol, Smoking and Substance Involvement Screening Test (ASSIST)

Multi-Substance Use / Screening

Purpose / Clinical Use

- Screens for risky use of ALL substance categories
- WHO-developed for primary care and general health settings
- Linked to brief intervention (ASSIST-BI)
- Identifies level of risk for each substance separately

Structure

- **Items:** 8 questions asked for EACH substance
- **Substances assessed (10 categories):** Tobacco, Alcohol, Cannabis, Cocaine, Amphetamines, Inhalants, Sedatives, Hallucinogens, Opioids, Other
- **Questions cover:** Lifetime use, frequency (past 3 months), urge to use, problems caused, failed role obligations, concern expressed by others, attempts to cut down, injection use
- **Response format:** Varies per question (frequency-based or Yes/No)

Scoring Method

- **Step 1:** Q1 (lifetime use) is a gateway question — if "No" for a substance, skip remaining questions for that substance
- **Step 2:** For each substance endorsed, sum Q2–Q7 scores
- **Step 3:** Calculate Substance-Specific Involvement Score (SSIS) for each substance
- **Q8 (injection):** Scored separately; not included in SSIS
- **Total involvement score:** Sum of all substance scores (for overall risk)

SSIS Range	Alcohol	All Other Substances	Risk Level
0–10	0–10	0–3	Low risk
11–26	11–26	4–26	Moderate risk (Brief Intervention)

027

027

027

High risk (Intensive treatment referral)

Injection drug use (Q8): Any injection in past 3 months = automatic referral regardless of score.

Clinical Interpretation

- **Low risk (0–3/0–10):** No intervention needed; brief positive feedback
- **Moderate risk:** Brief intervention (ASSIST-linked BI); psychoeducation; motivational interviewing
- **High risk (027):** Likely dependence; needs specialist referral
- **Multiple substances elevated:** Polysubstance use; complex treatment needs

Administration Details

- **Type:** Interview-administered (preferred) or self-report
- **Time:** 5–10 minutes
- **Age:** Adults (18+)
- **Available in:** Multiple languages including Hindi

Advantages

- Covers ALL substance categories in one instrument
- WHO-developed; globally validated
- Free to use
- Directly linked to intervention framework
- Efficient — skips irrelevant substances

Limitations

- Screening only — not diagnostic
- Self-report bias (underreporting)
- Different cut-offs for alcohol vs. other substances can be confusing
- May be too brief for comprehensive substance use assessment

Alternatives / Similar Scales

- **AUDIT:** Alcohol-only; more detailed for alcohol; same risk framework

- **DAST:** Drug-only (excluding alcohol); 20/28 items
- **CAGE-AID:** 4-item; adapted to include drugs; ultra-brief
- **TAPS:** Tobacco, Alcohol, Prescription, Substance; 2-step screening

19. Drug Abuse Screening Test (DAST-20 / DAST-10)

Drug Use (Non-Alcohol)

Purpose / Clinical Use

- Screens for drug use problems (excluding alcohol & tobacco)
- Quantifies degree of drug-related consequences
- Brief screening in clinical settings
- Identifies need for further assessment

Structure

- **Versions:** DAST-28, DAST-20, DAST-10
- **DAST-20:** 20 items (most commonly used)
- **DAST-10:** 10 items (brief version)
- **Response format:** Yes (1) / No (0)
- **Time frame:** Past 12 months
- **Covers:** Drug use patterns, consequences, loss of control, social problems, medical issues

Scoring Method

- **Step 1:** Score each item Yes = 1, No = 0
- **Step 2:** Reverse score items 4 & 5 (DAST-20): "No" = 1; "Yes" = 0
- **Step 3:** Sum all items
- **Score range (DAST-20):** 0–20
- **Score range (DAST-10):** 0–10

DAST-20 Score	DAST-10 Score	Severity / Action
0	0	No problems reported
1–5	1–2	Low level (Monitor, brief advice)
6–10	3–5	Moderate (Brief intervention)

11–15	6–8	Substantial (Intensive assessment)
16–20	9–10	Severe (Intensive treatment)

Clinical Interpretation

- **Score ≥6 (DAST-20):** Significant drug problem; needs comprehensive assessment
- **Score ≥3 (DAST-10):** Moderate risk; brief intervention indicated
- **High scores (≥16):** Likely substance use disorder; needs specialist treatment
- **Note:** Does not specify which drug(s) are problematic — follow-up needed

Administration Details

- **Type:** Self-report or interview
- **Time:** 5 minutes (DAST-10); 10 minutes (DAST-20)
- **Age:** Adults and adolescents

Advantages

- Brief and easy to administer
- Good psychometric properties
- Free to use
- Widely validated
- Useful in primary care screening

Limitations

- Does not identify specific drugs used
- Yes/No format lacks nuance
- Does not assess quantity/frequency
- Reverse scoring can cause errors
- Excludes alcohol and tobacco

Alternatives / Similar Scales

- **ASSIST:** Covers all substances; identifies which substance is problematic
- **AUDIT:** Alcohol-specific; complementary to DAST
- **CAGE-AID:** 4-item; quick screen; adapted for drugs

- **CRAFFT:** Adolescent-specific substance screening (6 items)

I. WITHDRAWAL SCALES

20. Clinical Institute Withdrawal Assessment for Alcohol - Revised (CIWA-Ar)

Alcohol Withdrawal

Purpose / Clinical Use

- Quantifies severity of alcohol withdrawal syndrome
- Guides pharmacological management (symptom-triggered therapy)
- Monitors withdrawal progression and treatment response
- Standard tool in deaddiction and emergency settings

Structure

- **Items:** 10 items
- **Response format:** Clinician-rated; items scored 0–7 (most items) or 0–4
- **Items (with max scores):**
 1. Nausea/Vomiting (0–7)
 2. Tremor (0–7)
 3. Paroxysmal Sweats (0–7)
 4. Anxiety (0–7)
 5. Agitation (0–7)
 6. Tactile Disturbances (0–7)
 7. Auditory Disturbances (0–7)
 8. Visual Disturbances (0–7)
 9. Headache/Fullness in Head (0–7)
 10. Orientation/Clouding of Sensorium (0–4)

Scoring Method

- **Step 1:** Rate each item based on observation and patient report
- **Step 2:** Sum all 10 items
- **Score range:** 0–67
- **Frequency of assessment:** Every 1–2 hours during acute withdrawal

Score	Severity	Management
≤8	Mild withdrawal	May not require medication; monitor
9–15	Moderate withdrawal	Consider benzodiazepines (symptom-triggered)
16–20	Severe withdrawal	Benzodiazepines indicated; close monitoring
>20	Very severe / Risk of seizures & DT	Aggressive treatment; ICU consideration

Clinical decision: Score ≤10 typically triggers benzodiazepine administration in symptom-triggered protocols.

Clinical Interpretation

- **Score >20:** High risk of delirium tremens & seizures; medical emergency
- **Rising scores:** Worsening withdrawal; increase monitoring frequency
- **Items 6–8 elevated:** Perceptual disturbances; impending delirium tremens
- **Red flags:** Score >20, disorientation (item 10 ≥3), hallucinations = ICU transfer

Administration Details

- **Type:** Clinician-rated (nurse/physician)
- **Time:** 2–5 minutes per assessment
- **Frequency:** Every 1–4 hours during withdrawal
- **Setting:** Inpatient, emergency, deaddiction units

Advantages

- Quick bedside assessment
- Guides symptom-triggered benzodiazepine therapy
- Reduces unnecessary medication use
- Well-validated; widely used globally
- Free to use

Limitations

- Requires patient cooperation (difficult if severely agitated/delirious)
- Subjective items (anxiety, headache) depend on patient report
- Not suitable for polysubstance withdrawal assessment
- Does not predict seizures with certainty

Alternatives / Similar Scales

- **AWS (Alcohol Withdrawal Scale):** Simpler; fewer items; used in some settings
- **PAWSS:** Prediction of Alcohol Withdrawal Severity Scale; predicts risk before withdrawal begins
- **CIWA-B:** Benzodiazepine withdrawal version
- **SHOT (Sweating, Hallucinations, Orientation, Tremor):** 4-item ultra-brief assessment

21. Clinical Opiate Withdrawal Scale (COWS)

Opioid Withdrawal

Purpose / Clinical Use

- Assesses severity of opioid withdrawal
- Guides buprenorphine/methadone induction timing
- Monitors withdrawal progression during detoxification
- Standard in opioid substitution therapy programs

Structure

- Items:** 11 items
- Response format:** Clinician-rated; items have varying scales (0–4 or 0–5)
- Items assessed:** Resting pulse rate, Sweating, Restlessness, Pupil size, Bone/joint aches, Runny nose/tearing, GI upset, Tremor, Yawning, Anxiety/irritability, Gooseflesh skin
- Based on:** Objective signs + patient report

Scoring Method

- Step 1:** Rate each item based on clinician observation and patient interview
- Step 2:** Sum all 11 items
- Score range:** 0–48

Score	Severity	Clinical Action
5–12	Mild withdrawal	Continue monitoring; consider symptomatic treatment
13–24	Moderate withdrawal	Buprenorphine induction appropriate
25–36	Moderately severe	Active treatment; buprenorphine/methadone
>36	Severe withdrawal	Aggressive treatment; close monitoring

Buprenorphine induction: Typically initiated when COWS score ≥13 (moderate withdrawal) to avoid precipitated withdrawal.

Clinical Interpretation

- **Score ≥ 13 :** Safe to initiate buprenorphine (sufficient withdrawal established)
- **Score <5 :** Not in withdrawal; do NOT give buprenorphine (risk of precipitated withdrawal)
- **Rising scores after buprenorphine:** May indicate precipitated withdrawal
- **Red flag:** Pulse >120 + severe GI symptoms + extreme anxiety = medical attention needed

Administration Details

- **Type:** Clinician-rated (observation + interview)
- **Time:** 2–5 minutes
- **Frequency:** Every 30–60 minutes during induction; then every 4–8 hours
- **Setting:** Deaddiction units, OST clinics, emergency departments

Advantages

- Quick and objective (relies on observable signs)
- Guides safe buprenorphine induction
- Simple scoring
- Free to use
- Widely validated

Limitations

- Some items subjective (anxiety, bone pain)
- Pulse may be elevated for other reasons (anxiety, medical conditions)
- Not validated for neonatal withdrawal (use Finnegan scale instead)
- Less useful for long-acting opioid withdrawal (delayed onset)

Alternatives / Similar Scales

- **SOWS (Subjective Opiate Withdrawal Scale):** Self-report; 16 items; patient perspective
- **OOWS (Objective Opiate Withdrawal Scale):** 13 objective signs only; observer-rated
- **Finnegan Scale (NAS):** Neonatal abstinence syndrome scoring; for newborns

- **CIWA-Ar:** For alcohol withdrawal (different pharmacology); not interchangeable

J. BELIEF ASSESSMENT SCALE

22. Brown Assessment of Beliefs Scale (BABS)

Insight / Delusional Conviction

Purpose / Clinical Use

- Assesses degree of conviction and insight regarding a specific belief
- Used across disorders: OCD (overvalued ideation), BDD, delusional disorder, psychosis
- Differentiates good insight vs. poor insight vs. delusional conviction
- Critical for treatment planning (poor insight = poorer CBT response)

Structure

- **Items:** 7 items
- **Response format:** Clinician-rated, 5-point scale (0–4) per item
- **Items assess:**
 1. Conviction (how certain the belief is true)
 2. Perception of others' views (do others share the belief?)
 3. Explanation of differing views (why others disagree)
 4. Fixity of ideas (willingness to consider alternatives)
 5. Attempt to disprove beliefs (reality testing efforts)
 6. Insight (recognition that belief may be psychiatric symptom)
 7. Ideas/delusions of reference (NOT included in total score)
- **Target belief:** Identified collaboratively before administration (e.g., the primary obsession in OCD)

Scoring Method

- **Step 1:** Identify the target belief to be assessed
- **Step 2:** Rate items 1–6 on scale of 0–4
- **Step 3:** Sum items 1–6 (Item 7 is NOT included in total)
- **Total score range:** 0–24
- **Higher scores = poorer insight / greater delusional conviction**

Total Score	Level of Insight
0–3	Good insight (recognizes belief is likely not true)
4–7	Fair insight
8–12	Poor insight
13–17	Absent insight / Overvalued ideation
18–24	Delusional conviction

Clinical threshold: Score ≥ 18 = beliefs held with delusional intensity (consider "OCD with absent insight" specifier per DSM-5).

Clinical Interpretation

- **Score 0–7:** Good to fair insight; likely to benefit from standard CBT/ERP
- **Score 8–12:** Poor insight; may need modified CBT approach; consider augmentation strategies
- **Score ≥ 13 :** Absent insight/delusional; poor prognosis with CBT alone; antipsychotic augmentation may be needed
- **Use with Y-BOCS:** High Y-BOCS + High BABS = OCD with poor/absent insight (DSM-5 specifier)
- **Red flag:** Score ≥ 18 suggests beliefs are held with delusional intensity; differentiate from primary delusional disorder

Administration Details

- **Type:** Clinician-rated (semi-structured interview)
- **Time:** 10–15 minutes
- **Age:** Adults; adolescents (with adaptation)
- **Prerequisite:** Target belief must be identified before administration

Advantages

- Only validated scale specifically for belief assessment/insight across disorders
- Useful in OCD, BDD, eating disorders, delusional disorder
- Brief administration

- Good psychometric properties
- Free to use
- Maps to DSM-5 insight specifier for OCD

Limitations

- Assesses only ONE belief at a time
- Requires clinical judgment to select target belief
- Limited validation in psychosis populations (developed for OCD/BDD)
- May fluctuate with mood state

Alternatives / Similar Scales

- **Y-BOCS Item 11 (Insight)**: Single item; less detailed; part of Y-BOCS
- **OVIS (Overvalued Ideas Scale)**: 11 items; assesses overvalued ideation specifically
- **David's Insight Scale (SAI)**: 3 dimensions of insight; for psychosis
- **BCIS (Beck Cognitive Insight Scale)**: Self-report; measures cognitive insight in psychosis

K. SUICIDE ASSESSMENT SCALE

23. Columbia-Suicide Severity Rating Scale (C-SSRS)

Suicide Risk Assessment

Purpose / Clinical Use

- Gold standard for assessing suicidal ideation and behavior
- Prospective risk assessment and monitoring
- Distinguishes suicidal ideation from suicidal behavior (critical distinction)
- Used in clinical settings, emergency departments, research, and FDA-mandated drug trials
- Available for **adults** and **children/adolescents (age 6+)**

Structure

- **Two main sections:**

Section 1: Suicidal Ideation (5 items) — rated for severity

1. Wish to be dead
2. Non-specific active suicidal thoughts
3. Active suicidal ideation with any methods (not plan) without intent
4. Active suicidal ideation with some intent to act, without specific plan
5. Active suicidal ideation with specific plan and intent

Intensity rating (for most severe ideation): Frequency, Duration, Controllability, Deterrents, Reasons (each 1–5)

Section 2: Suicidal Behavior (5 categories)

1. Actual attempt
2. Interrupted attempt
3. Aborted attempt
4. Preparatory acts/behavior
5. Non-suicidal self-injurious behavior (NSSI)

- **Response format:** Yes/No for presence; severity rated on ordinal categories
- **Versions:** Lifetime/Recent, Since Last Visit, Baseline/Screening, Children's version (age 6–11)

Scoring Method

- **Step 1:** Determine highest level of suicidal ideation endorsed (1–5)
- **Step 2:** Rate intensity of most severe ideation (5 subscales, each 1–5; total intensity range: 5–25)
- **Step 3:** Determine suicidal behavior category (if any)
- **Step 4:** Rate lethality of any attempts (0–5 scale + medical damage)
- **No single total score** — interpreted categorically

Ideation Severity	Category	Risk Level
0	No ideation	Low
1	Wish to be dead	Low-Moderate
2	Non-specific active thoughts	Moderate
3	Active ideation with methods	High
4	Ideation with intent, no plan	High
5	Ideation with plan and intent	Very High / Emergency

Behavior Category	Action Required
Preparatory acts	Immediate safety planning; consider hospitalization
Aborted/Interrupted attempt	Urgent psychiatric evaluation
Actual attempt	Emergency intervention; hospitalization

Children's Version (C-SSRS Children's)

- **Age:** 6–11 years (simplified language)
- **Same structure** but developmentally appropriate wording
- **Key considerations:** Children may not fully understand death's permanence; assess cognitive maturity
- **Collateral information:** Always supplement with parent/caregiver interview

Clinical Interpretation

- **Ideation level 4–5:** Imminent risk; requires immediate intervention
- **Any suicidal behavior (actual/interrupted/aborted):** High-risk; safety planning mandatory
- **High intensity scores:** Frequent, uncontrollable thoughts with few deterrents = highest concern
- **Red flags:** Ideation level ≥ 4 , any behavior category endorsed, high lethality, low controllability, absence of deterrents

Administration Details

- **Type:** Clinician-administered (structured interview)
- **Time:** 5–10 minutes
- **Age:** Adults (18+); Children/Adolescents (6–17 with adapted version)
- **Training:** Free online training available (cssrs.columbia.edu)
- **Setting:** Clinical, emergency, research, schools, military

Advantages

- FDA gold standard for clinical trials
- Clear distinction between ideation and behavior
- Free to use (public domain)
- Validated across populations (adults, adolescents, children)
- Multiple versions for different timeframes
- Categorical risk assessment aids clinical decision-making
- Available in 140+ languages including Hindi

Limitations

- No single summary score (can be complex to interpret)
- Relies on patient disclosure (concealment possible)
- Young children may not comprehend abstract questions about death
- Snapshot in time — suicidality fluctuates rapidly
- Does not replace comprehensive suicide risk assessment

Alternatives / Similar Scales

- **PHQ-9 Item 9:** Single screening item for suicidal thoughts; very brief; low specificity
- **Beck Scale for Suicidal Ideation (BSS):** 21-item; self-report; quantifies ideation severity
- **SAD PERSONS Scale:** 10-item mnemonic-based risk assessment; clinical decision tool
- **SBQ-R (Suicidal Behaviors Questionnaire-Revised):** 4-item self-report; lifetime + recent

L. CRAVING & SUBSTANCE-SPECIFIC SCALES

24. Obsessive Compulsive Drug Use Scale (OCDUS)

Craving / Substance Use

Purpose / Clinical Use

- Measures intensity and intrusiveness of drug craving using OCD-like dimensions
- Conceptualizes craving as obsessive thoughts and compulsive drug-seeking behavior
- Monitors craving severity during treatment and recovery
- Originally developed for heroin; adapted versions for alcohol (OCDS), cocaine, nicotine

Structure

- **Items:** 12 items (OCDUS for drugs); OCDS = 14 items (for alcohol)
- **3 Subscales:**
 - 1. **Thoughts & Interference:** Frequency, intrusiveness, distress of drug thoughts (items 1–5)
 - 2. **Desire & Control:** Intensity of desire, ability to resist (items 6–9)
 - 3. **Resistance:** Efforts to resist drug thoughts and use (items 10–12)
- **Response format:** 5-point Likert scale (0–4) per item
- **Time frame:** Past 7 days

Scoring Method

- **Step 1:** Rate each item 0–4
- **Step 2:** Sum all items for total score
- **Total score range (OCDUS-12):** 0–48
- **Subscale scores:** Sum relevant items for each subscale
- **Higher scores = more intense craving**

Total Score	Craving Severity
0–10	Minimal craving

11–20	Mild craving
21–30	Moderate craving
31–40	Severe craving
41–48	Extreme craving

OCDS (Alcohol version): 14 items; Score range 0–56; Cut-off ≥ 20 suggests clinically significant craving.

Clinical Interpretation

- **High Thoughts & Interference:** Intrusive drug thoughts dominating cognition; poor concentration
- **High Desire & Control:** Intense urges with poor ability to resist; high relapse risk
- **High Resistance (low):** Patient not actively fighting craving; motivational work needed
- **Increasing scores over time:** Relapse risk increasing; intensify intervention
- **Red flag:** Total ≥ 30 + declining resistance = imminent relapse risk

Administration Details

- **Type:** Self-report or clinician-administered
- **Time:** 5–10 minutes
- **Age:** Adults (18+)
- **Versions:** OCDUS (drugs), OCDS (alcohol), OCDCS (cocaine), OCDNS (nicotine)

Advantages

- Captures cognitive dimension of craving (obsessive quality)
- Multidimensional (thoughts + desire + resistance)
- Sensitive to treatment change
- Available for multiple substances
- Good psychometric properties
- Theoretically grounded (OCD model of addiction)

Limitations

- No universally established clinical cut-offs
- Self-report bias (denial, minimization)
- Different versions for different substances (not one unified tool)
- May not capture impulsive/cue-triggered craving well

Alternatives / Similar Scales

- **DDQ (Desires for Drug Questionnaire)**: 13 items; 3 factors; broader craving model
- **ACQ (Alcohol Craving Questionnaire)**: 47/12 items; multidimensional craving
- **Penn Alcohol Craving Scale (PACS)**: 5 items; brief; frequency + intensity + duration
- **VAS (Visual Analogue Scale for Craving)**: Single item 0–100mm; ultra-brief; less reliable

M. ADDITIONAL HIGH-YIELD SCALES

25. Global Assessment of Functioning (GAF) Scale

Global Functioning

Purpose / Clinical Use

- Rates overall psychological, social, and occupational functioning
- Part of DSM-IV multiaxial system (Axis V)
- Provides single global functioning score
- Still widely used in Indian clinical settings despite DSM-5 dropping it

Structure

- **Format:** Single continuous scale 0–100
- **Divided into 10-point intervals**
- **Higher score = better functioning**

Scoring Method

Score Range	Level of Functioning
91–100	Superior functioning; no symptoms
81–90	Absent or minimal symptoms; good functioning in all areas
71–80	Transient symptoms; slight social/occupational impairment
61–70	Mild symptoms; some difficulty in social/occupational functioning
51–60	Moderate symptoms; moderate difficulty in functioning
41–50	Serious symptoms; serious impairment in functioning
31–40	Major impairment in several areas; some impairment in reality testing
21–30	Behavior influenced by delusions/hallucinations; serious impairment
11–20	Some danger of hurting self/others; gross impairment in communication
1–10	Persistent danger; unable to maintain minimal personal hygiene

Administration Details

- **Type:** Clinician-rated
- **Time:** 1–2 minutes
- **Note:** Replaced by WHODAS 2.0 in DSM-5, but still used in practice

Alternatives / Similar Scales

- **WHODAS 2.0:** DSM-5 replacement; 36-item; disability-focused; WHO-developed
- **SOFAS:** Social and Occupational Functioning Scale; excludes symptom severity
- **PSP:** Personal and Social Performance; 4 domains; used in schizophrenia

26. Patient Health Questionnaire-9 (PHQ-9)

Depression Screening

Purpose / Clinical Use

- Brief depression screening and severity measure
- Directly maps to DSM-5 MDD criteria (9 criteria)
- Widely used in primary care and research
- Free, publicly available

Structure

- **Items:** 9 items (each = one DSM-5 criterion)
- **Response format:** 4-point Likert (0–3): Not at all, Several days, More than half the days, Nearly every day
- **Time frame:** Past 2 weeks
- **10th item (functional impairment):** Not scored but clinically informative

Scoring Method

- **Sum all 9 items:** Range 0–27
- **Diagnostic algorithm:** 5+ items scored ≥ 2 (including item 1 or 2) = probable MDD

Score	Severity	Action
0–4	Minimal/None	Monitor
5–9	Mild	Watchful waiting; repeat at follow-up
10–14	Moderate	Treatment plan; counseling or pharmacotherapy
15–19	Moderately Severe	Active treatment with pharmacotherapy &/or psychotherapy
20–27	Severe	Immediate treatment; specialist referral

Administration Details

- **Type:** Self-report
- **Time:** 2–3 minutes
- **Age:** Adolescents and adults
- **Free to use:** No copyright restrictions

Alternatives / Similar Scales

- **BDI-II:** More comprehensive (21 items); copyrighted; better for clinical populations
- **PHQ-2:** First 2 items of PHQ-9; ultra-brief screening; cut-off ≥ 3
- **HAM-D:** Clinician-rated; gold standard for research

27. Generalized Anxiety Disorder 7-item Scale (GAD-7)

Anxiety Screening

Purpose / Clinical Use

- Brief screening for generalized anxiety disorder
- Also sensitive to panic disorder, social anxiety, PTSD
- Monitoring treatment response
- Free, widely used in primary care

Structure

- **Items:** 7 items
- **Response format:** 4-point Likert (0–3): Not at all, Several days, More than half the days, Nearly every day
- **Time frame:** Past 2 weeks

Scoring Method

- **Sum all 7 items:** Range 0–21

Score	Severity
0–4	Minimal Anxiety
5–9	Mild Anxiety
10–14	Moderate Anxiety
15–21	Severe Anxiety

Cut-off ≥10: Good sensitivity (89%) and specificity (82%) for GAD diagnosis.

Administration Details

- **Type:** Self-report
- **Time:** 2–3 minutes
- **Free to use**

Alternatives / Similar Scales

- **BAI:** 21-item; somatic focus; copyrighted
- **HAM-A:** Clinician-rated; comprehensive; gold standard for trials
- **GAD-2:** First 2 items; ultra-brief screen; cut-off ≥ 3

28. Depression Anxiety Stress Scales - 21 (DASS-21)

Depression / Anxiety / Stress

Purpose / Clinical Use

- Simultaneously measures depression, anxiety, and stress
- Discriminates between three negative emotional states
- Screening and severity assessment
- Widely used in research and clinical settings in India

Structure

- **Items:** 21 items (7 per subscale)
- **3 Subscales (7 items each):**
 - Depression: Dysphoria, hopelessness, devaluation of life, self-deprecation, anhedonia, lack of interest, inertia
 - Anxiety: Autonomic arousal, skeletal muscle effects, situational anxiety, subjective anxiety
 - Stress: Nervous arousal, difficulty relaxing, irritability, impatience, overreaction
- **Response format:** 4-point scale (0–3): Did not apply, Applied to some degree, Applied a considerable degree, Applied very much/most of the time
- **Time frame:** Past 1 week

Scoring Method

- **Step 1:** Sum items for each subscale (7 items each)
- **Step 2:** MULTIPLY each subscale score by 2 (to match DASS-42 norms)
- **Score range per subscale (after ×2):** 0–42

Severity	Depression	Anxiety	Stress
Normal	0–9	0–7	0–14
Mild	10–13	8–9	15–18
Moderate	14–20	10–14	19–25

Severe	21–27	15–19	26–33
Extremely Severe	28+	20+	34+

CRITICAL: Must multiply raw subscale scores $\times 2$ before using cut-offs above!

Administration Details

- **Type:** Self-report
- **Time:** 5–10 minutes
- **Age:** 17+ years
- **Free to use** (public domain)

Advantages

- Three constructs in one instrument
- Free and publicly available
- Good discriminant validity between depression, anxiety, and stress
- Widely validated in Indian populations
- Useful for non-clinical and clinical samples

Limitations

- Must remember to multiply by 2 for DASS-21 (common error)
- Not diagnostic; screening only
- "Stress" construct not well-mapped to DSM categories
- Anxiety subscale heavily somatic (like BAI)

Alternatives / Similar Scales

- **PHQ-9 + GAD-7:** Separate tools for depression & anxiety; DSM-aligned
- **BDI-II + BAI:** Separate Beck tools; copyrighted; more detailed
- **K-10 (Kessler Psychological Distress Scale):** 10-item; general distress; widely used in epidemiology

QUICK REFERENCE: SCALE COMPARISON TABLE

Depression Scales at a Glance

Scale	Items	Type	Range	Cut-off (Moderate)	Free?
BDI-II	21	Self-report	0–63	≥20	No
HAM-D	17	Clinician	0–52	≥14	Yes
PHQ-9	9	Self-report	0–27	≥10	Yes
DASS-21 (D)	7	Self-report	0–42*	≥14*	Yes

*After multiplying by 2

Anxiety Scales at a Glance

Scale	Items	Type	Range	Cut-off (Moderate)	Free?
BAI	21	Self-report	0–63	≥16	No
HAM-A	14	Clinician	0–56	≥25	Yes
GAD-7	7	Self-report	0–21	≥10	Yes
DASS-21 (A)	7	Self-report	0–42*	≥10*	Yes

Psychosis Scales at a Glance

Scale	Items	Type	Range	Key Feature
BPRS	18	Clinician	18–126	General psychopathology; quick
PANSS	30	Clinician	30–210	P/N/G subscales; gold standard
SAPS	34	Clinician	0–170	Detailed positive symptoms only
SANS	25	Clinician	0–125	Detailed negative symptoms only

Cognitive Scales at a Glance

Scale	Max Score	Time	Executive Function?	Best For
MMSE	30	5–10 min	No	Dementia screening (bedside)
MoCA	30	10–15 min	Yes	MCI detection
ACE-III	100	15–20 min	Yes	Dementia type differentiation

Substance Use Scales at a Glance

Scale	Items	Substances	Range	Key Feature
AUDIT	10	Alcohol only	0–40	WHO; linked to brief intervention
ASSIST	8 per substance	All (10 categories)	0–39 per substance	Multi-substance; WHO
DAST-20	20	Drugs (not alcohol)	0–20	Drug-specific; Yes/No format
OCDUS	12	Drug craving	0–48	OCD model of craving

Withdrawal Scales at a Glance

Scale	Items	Range	Substance	Key Clinical Use
CIWA-Ar	10	0–67	Alcohol	Guides benzodiazepine dosing
COWS	11	0–48	Opioids	Guides buprenorphine induction (13)

